MenoVerdict

0 none

1 mild

2 moderate

3 severe

# Symptom Journal & Doctor Visit Toolkit

Track your perimenopause or menopause symptoms for eight weeks, see the pattern, and walk into your appointment with a clear one-page summary and the right questions.

Printable workbook, 38 pages

menoverdict.com

Edition 1, November 2026

# Please read this first

This workbook gathers what trusted medical organisations say about perimenopause, menopause and the years after. It is general information. It is not a diagnosis, and it can't replace a doctor who knows you.

### We are not doctors

MenoVerdict is not a medical provider. This workbook was researched and written by the MenoVerdict team from published sources, and it has not been reviewed by your doctor. Even where the information comes from doctors and health agencies, only a clinician who knows your history, examines you and runs the right tests can tell you what is causing your symptoms and what is safe for you.

## Where the information comes from

Government health agencies and medical organisations, including the US National Institutes of Health (NIH), the US Food and Drug Administration (FDA), the UK's National Health Service (NHS), The Menopause Society, Mayo Clinic and Cleveland Clinic. Every source is listed on the last page and was checked in November 2026. Medical guidance changes, so newer advice may replace what's here.

## What this workbook does

- **Describes symptoms many women commonly report** in each stage. Having a symptom on these pages doesn't mean you're in that stage, and the same symptoms can have other causes.
- **Describes treatments and home remedies people commonly use**, and what the evidence says about them. Describing a remedy is not recommending it.
- **Helps you track and explain your symptoms**, so your appointment is more useful.

## What we ask of you

- Don't start, stop or change any medicine, supplement or home remedy because of this workbook. Talk to your doctor or pharmacist first. Natural products can have side effects and interact with medicines.
- Don't use this workbook to rule out a serious problem. If something worries you, get it checked.
- For anything urgent, use the "Get help now" page and call 911 or your local emergency number.

By using this workbook, you agree that it provides general information only and that decisions about your health should be made with a qualified clinician. To the extent the law allows, MenoVerdict isn't responsible for decisions made using this workbook.

# How to use this toolkit

Symptoms are easy to forget and hard to describe in a short appointment. A few minutes of tracking a day turns "I just don't feel like myself" into a clear pattern your clinician can act on.

1. **Start with your snapshot**
   Fill in your health snapshot, medicines list and symptom checklist. It takes about 20 minutes, once.
2. **Track every day**
   Use the weekly tracker for at least two weeks, ideally eight. It takes two or three minutes each evening.
3. **Review each month**
   Use the monthly review to spot patterns: what's getting better, what's getting worse, and what seems to set things off.
4. **Prepare for your appointment**
   Complete the one-page summary, tick the questions you want to ask, and bring this workbook with you.
5. **Follow up**
   Write down the plan, then track how you feel on it, so your next appointment starts from facts.

## The heat scale

Rate each symptom the same way every day.

- 0 **None.** Not there today.
- 1 **Mild.** Noticeable, but doesn't change your day.
- 2 **Moderate.** Changes what you do or how well you do it.
- 3 **Severe.** Stops you, or badly disrupts sleep or work.

## Printing and filling it in

Print at home on US Letter paper at 100% scale. Print the weekly trackers as many times as you need.

Prefer a tablet? Open the PDF in a note app that lets you write on PDFs, and fill it in with a stylus or your finger.

The coloured tabs on the right edge mark the three sections: Learn, Track and Prepare.

# What's inside

Three sections, in the order most women use them.

**Learn**
The stages and typical ages, symptoms by stage, other possible causes, home remedies people try and what the evidence says, and treatment options.

**Track**
Your health snapshot, symptom checklist, bleeding calendar, eight weekly trackers, logs and monthly reviews.

**Prepare**
Your appointment plan, a one-page summary for your clinician, questions to ask, a script, and notes for after the visit.

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## Learn


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# Get help now

Tracking is for everyday symptoms. These signs need a professional, not a journal.

## Call 911 or your local emergency number

- Chest pain, pressure or tightness
- Fainting, or a racing heartbeat with dizziness or shortness of breath
- Sudden weakness or numbness, a drooping face, or trouble speaking
- A sudden, severe headache unlike your usual headaches
- Very heavy bleeding with dizziness or feeling faint

### If you're thinking about harming yourself

In the US, call or text **988** (Suicide & Crisis Lifeline) at any time. Outside the US, call your local emergency number or crisis line.

## See a doctor soon, within days rather than months

- **Any vaginal bleeding after 12 months without a period**, even once and even spotting. It's usually not serious, but it needs checking. [1]
- **Bleeding that's getting heavier** rather than lighter, or soaking at least one pad or tampon an hour for more than two hours in a row. [1] [2]
- **Irregular or heavier bleeding after six months on hormone therapy.** [1]
- **A fast or pounding heartbeat** (palpitations). [1]
- **If you take fezolinetant (Veozah):** yellow skin or eyes, dark urine, pale stools, unusual itching, nausea or pain in the upper right belly. The FDA advises stopping it and contacting your prescriber straight away. [3]

### About this toolkit

This workbook is for general information and personal tracking. It isn't medical advice, a diagnosis or a treatment plan, and MenoVerdict isn't a medical provider. Talk to a qualified clinician about your own symptoms and treatment. Numbered sources are listed on the last page.

# The three stages at a glance

Menopause isn't a long stage. It's a single point in time: 12 months after your last period. The years before it are perimenopause, and everything after it is postmenopause.

## Perimenopause

**Usually starts between 45 and 55**

Hormone levels rise and fall unevenly. Periods start to change, and symptoms often begin.

**Common:** irregular periods, hot flashes, night sweats, sleep problems, mood changes, brain fog.

## Menopause

**Average age in the US: 52**

The day you reach 12 months in a row without a period. You only know it looking back.

## Postmenopause

**For the rest of your life**

Your ovaries no longer release eggs. Some symptoms ease, while others can continue for years.

**Common:** vaginal dryness, bladder changes, ongoing hot flashes for some, faster bone loss.

Sources [9] [10] [11]

## How long symptoms last

The NIH says symptoms related to menopause can last between two and eight years. [9] A large US study found frequent hot flashes and night sweats lasted a median of 7.4 years in total, and longer for women whose symptoms started early in the transition. [12] Everyone is different, so your own experience may be shorter or longer.

## Earlier than usual

**Early menopause** means periods stop before age 45, and **premature menopause** means before age 40. [13] Possible causes include family history, smoking, cancer treatment and surgery to remove the ovaries, which causes menopause straight away. [9] [13] If your periods stop before 45, see a doctor to talk about what it means for your health.

## A rough guide to where you might be

| If this sounds like you | You might be |
|---|---|
| **Periods still regular, with new symptoms like hot flashes or poor sleep** | In early perimenopause, or something else may be causing your symptoms. |
| **Periods closer together, further apart or skipped** | In perimenopause, especially if you're in your 40s or 50s. |
| **No period for 12 months in a row** | Postmenopausal. You've reached menopause. |
| **Hysterectomy, hormonal IUD, implant or birth control pill** | Hard to tell from periods. Track your symptoms and ask your doctor. |

A rough guide only. Only your clinician can confirm your stage.

# Perimenopause: the transition

The years when your body moves towards menopause. It often starts in your mid-40s, but can start earlier. Many symptoms can come and go.

## Symptoms women commonly report

- Changes to periods: more or less often, heavier or lighter
- Hot flashes and night sweats
- Trouble sleeping
- Low mood, anxiety, irritability or mood swings
- Brain fog, poor memory or concentration
- Headaches, or worse migraines
- Fast or pounding heartbeat (palpitations)
- Joint and muscle aches
- Weight gain, often around the middle
- Vaginal dryness, itching or pain during sex
- Needing to pee more often, or UTI-like symptoms
- Lower sex drive
- Thinning hair, dry or itchy skin
- Sensitive teeth or sore gums

Symptom list from the NHS [1] and NIH [9]. You may have a few of these, many, or none.

## Things worth knowing

### You can still get pregnant

Until you're postmenopausal, pregnancy is possible even if your periods are irregular. NHS guidance advises continuing contraception for two years after your last period if you're 40 to 50, or one year after if you're over 50. [14]

### Your periods will change

Irregular periods are expected. Bleeding that's getting heavier rather than lighter, or very heavy bleeding, should be checked. See "Get help now". [1] [2]

### Many symptoms have other causes too

Thyroid problems, low iron, depression, sleep apnea and some medicines can all cause similar symptoms. See "Could it be something else?".

## My notes

# Menopause and the years after

Once you've gone 12 months without a period, you've reached menopause and you're postmenopausal from then on. Pregnancy is no longer possible. [11]

## Symptoms that can continue or start

Many women have lingering symptoms that are usually milder, while some have none. [11]

- Hot flashes and night sweats
- Vaginal dryness and pain during sex
- Leaking urine or bladder changes
- Trouble sleeping
- Low mood
- Lower sex drive
- Dry skin and hair loss
- Weight changes

Hot flashes that are bothersome or getting worse after menopause should be checked by a clinician, to rule out other causes. [11]

## Long-term health changes

| Change | What happens | Worth asking |
|---|---|---|
| **Bones** | Bone density drops faster after menopause because of lower estrogen, which raises the risk of osteoporosis and fractures. [11] | Do I need a bone density scan? |
| **Heart and circulation** | The risk of heart disease and stroke rises after menopause. [9] [11] | Which heart checks do I need? |
| **Vagina and bladder** | Lower estrogen makes vaginal tissue thinner and drier, and can affect the bladder. This is called genitourinary syndrome of menopause. [11] | What would help my dryness or bladder symptoms? |

### Bleeding after menopause is never normal to ignore

Any vaginal bleeding after 12 months without a period should be checked by a doctor, even if it happens once. It's usually not serious, but it can be a sign of cancer. [1]

# Could it be something else?

Many menopause symptoms are shared with other conditions, and some of those need treatment. This is why a doctor may suggest tests, even when menopause seems the obvious answer.

| If you have | Other possible causes | Why it matters |
|---|---|---|
| **Hot flashes or night sweats** | An overactive thyroid, infections, diabetes, sleep apnea, anxiety or panic, alcohol, pregnancy, and some medicines, including some blood pressure drugs, antidepressants and steroids. [15] | Night sweats that disrupt your sleep, or come with other symptoms, are worth a visit. [15] |
| **Tiredness, low mood, hair loss, weight changes or poor sleep** | Thyroid disorders share many of these symptoms with menopause. [16] Depression can cause several of them too. | A simple blood test can rule out a thyroid problem. [16] |
| **Tiredness or breathlessness with heavy periods** | Blood loss from heavy bleeding can leave you tired, weak or short of breath. [2] | Heavy bleeding needs checking, and low iron can be treated. |
| **Fast or pounding heartbeat** | Palpitations can have causes other than menopause. | The NHS advises seeing a GP about palpitations. [1] Get emergency help if they come with chest pain or fainting. |
| **Irregular or heavier bleeding** | Pregnancy is still possible in perimenopause. [14] Other conditions can also cause heavier bleeding. | Heavier, not lighter, bleeding should be checked. [1] |
| **Bleeding after menopause** | Usually not serious, but it can be a sign of cancer. [1] | Always get it checked. |
| **Burning when peeing, or needing to go urgently** | Menopause can cause UTI-like symptoms, but so can a real infection. [1] | A urine test can tell the difference. |

**Only a clinician can work out what's causing your symptoms.** Bring your tracker and your medicines list, because both help rule causes in or out.

# Home remedies people try

**We don't recommend trying any of these without talking to your doctor or pharmacist first.** "Natural" doesn't mean safe. Supplements can have side effects, contain unlisted ingredients and interact with your medicines. [17] [18] This page describes what people commonly use and what the evidence says, nothing more.

| What people try | Used for | What the evidence says | Safety notes |
|---|---|---|---|
| **Black cohosh** | Hot flashes | Mixed. A 2023 review suggested possible benefit for hot flashes, but not for mood. [18] An expert panel didn't recommend supplements for hot flashes. [4] | Rare cases of serious liver damage reported. Some products contained the wrong herb. [18] |
| **Soy and soy isoflavones** | Hot flashes | Little evidence they help. [17] Not recommended by an expert panel. [4] | Check with your doctor if you take other medicines. |
| **Red clover, dong quai, kava** | Hot flashes | No conclusive evidence they reduce hot flashes. [17] | Can interact with medicines. [17] |
| **DHEA** | Energy, sex drive | Little evidence it helps. [17] | Safety concerns, because the body turns it into hormones. [17] |
| **Cannabis or CBD** | Sleep, hot flashes | Not recommended by an expert panel. [4] | Legal status and product quality vary. |
| **Acupuncture** | Hot flashes | Not recommended by an expert panel. [4] | Use a licensed practitioner if you try it. |
| **Yoga, tai chi, meditation** | Stress, sleep, hot flashes | Tai chi and meditation programs may help common symptoms like sleep, mood and stress. [17] Yoga and mindfulness weren't recommended for hot flashes specifically. [4] | Generally low risk. Good for overall wellbeing. |
| **Clinical hypnosis** | Hot flashes | Recommended by an expert panel, with some supporting evidence. [4] [17] | Look for a trained practitioner. |
| **Cooling: fans, layers, a cool bedroom** | Comfort during hot flashes | Not shown to reduce how often hot flashes happen, though some women find them helpful for comfort. [4] | Low risk. |
| **Avoiding alcohol, caffeine, spicy food** | Hot flashes | Not recommended as a treatment, because the evidence is weak. [4] Tracking can show your own pattern. | Cutting down on alcohol is good for your health anyway. |
| **Vaginal moisturisers and lubricants** | Dryness, painful sex | A common first step for vaginal symptoms. [5] | Choose products made for intimate use. |

# Know your options

A plain summary of what current expert guidance says, so you can have an informed conversation. It's not a recommendation for you. Your clinician will weigh your history and preferences.

| Option | What the evidence says | Worth asking |
|---|---|---|
| **Hormone therapy** | The Menopause Society calls it the most effective treatment for hot flashes, night sweats and vaginal symptoms. For most healthy women under 60, or within 10 years of menopause, it says the benefits outweigh the risks. [5] In February 2026 the FDA approved label changes removing heart disease, breast cancer and dementia statements from the boxed warning. [6] Women who still have a uterus are usually prescribed a progestogen with estrogen to protect the womb lining. | Am I a good candidate? Which form suits me (patch, gel, spray or pill)? What are my own risks in actual numbers? |
| **Non-hormonal prescription medicines** | Recommended options for hot flashes include fezolinetant (Veozah), certain antidepressants (SSRIs and SNRIs), gabapentin and oxybutynin. [4] Fezolinetant needs liver blood tests before and during treatment. [3] Elinzanetant (Lynkuet), a newer option, was approved by the FDA in October 2025. [7] | Which would suit me if I can't or don't want to take hormones? What monitoring would I need? |
| **Talking and mind-body therapies** | Cognitive behavioural therapy (CBT) and clinical hypnosis are recommended for hot flashes. [4] UK guidance also recommends menopause-specific CBT for hot flushes, night sweats and sleep problems, alongside or instead of hormone therapy. [8] | Where can I get menopause-specific CBT or clinical hypnosis? Is it covered? |
| **Vaginal dryness and painful sex** | Non-hormonal vaginal moisturisers and lubricants are a common first step. When they aren't enough, low-dose vaginal estrogen appears safe and greatly improves quality of life for many women. [5] | Would vaginal estrogen suit me? Can I use it with other treatment? |
| **Bladder leaks** | Leaks have several possible causes, and the right treatment depends on the type. | What type of leaking do I have? Would pelvic floor physiotherapy help? |
| **Not recommended for hot flashes** | An expert panel did not recommend supplements and herbal remedies, soy, cannabinoids, acupuncture, paced breathing, yoga, mindfulness, cooling techniques or avoiding triggers as treatments for hot flashes, because the evidence is weak or mixed. [4] Exercise and good sleep habits are still good for your overall health, and cooling can make an episode more comfortable. | If I'm taking a supplement, could it interact with my medicines? |

# My health snapshot

The background your clinician will ask about. Fill it in once and update it when something changes.

Name _____ Date started _____

Age _____ Height and weight (optional) _____



## My periods

Age when periods started _____ First day of last period _____

- ☐ Periods are regular
- ☐ Periods are irregular
- ☐ Periods have stopped
- ☐ Not sure
- ☐ I have a hormonal IUD or implant
- ☐ I take hormonal birth control
- ☐ I've had a hysterectomy
- ☐ My ovaries have been removed

How have your periods changed in the last year? _____

## Have you ever had any of these?

These matter when you talk about treatment options. Tick any that apply to you.

- ☐ Blood clot in a leg or lung
- ☐ Stroke or heart attack
- ☐ Breast cancer
- ☐ Womb (endometrial) or ovarian cancer
- ☐ Liver disease
- ☐ Migraine with aura
- ☐ Osteoporosis or a fracture from a minor fall
- ☐ Endometriosis or fibroids
- ☐ Depression or anxiety
- ☐ Thyroid problems

Other conditions or surgeries _____

## Family history

- ☐ Breast or ovarian cancer
- ☐ Heart disease before age 60
- ☐ Osteoporosis or hip fracture
- ☐ Early menopause (before 45)

Who, and at what age? _____

## Day to day

Smoking or vaping _____ Alcoholic drinks per week _____

Exercise per week _____ Current birth control _____

# Medicines and supplements

List everything you take, including vitamins, herbal products and anything bought without a prescription.

| Name | Dose and how often | Started | What it's for |
|---|---|---|---|
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Allergies or reactions to medicines _____

## My care team

| Role | Name and practice | Phone | Patient portal or email |
|---|---|---|---|
| Primary care | | | |
| Gynecologist | | | |
| Menopause specialist | | | |
| Pharmacy | | | |
| | | | |

Insurance plan _____ Member ID _____

# Where I'm starting

Rate every symptom on day one, even the ones that seem unrelated. Many women don't realise how many of their symptoms can be linked to perimenopause, and many have other causes too.

| Symptom | Heat scale today | How often | Notes |
|---|---|---|---|
| **Heat and heartbeat** | | | |
| Hot flashes | 0 1 2 3 | rarely weekly daily | |
| Night sweats | 0 1 2 3 | rarely weekly daily | |
| Chills | 0 1 2 3 | rarely weekly daily | |
| Fast or pounding heartbeat | 0 1 2 3 | rarely weekly daily | |
| **Sleep and energy** | | | |
| Trouble falling asleep | 0 1 2 3 | rarely weekly daily | |
| Waking during the night | 0 1 2 3 | rarely weekly daily | |
| Waking too early | 0 1 2 3 | rarely weekly daily | |
| Tiredness or low energy | 0 1 2 3 | rarely weekly daily | |
| **Mood and mind** | | | |
| Low mood | 0 1 2 3 | rarely weekly daily | |
| Anxiety or panic | 0 1 2 3 | rarely weekly daily | |
| Irritability or anger | 0 1 2 3 | rarely weekly daily | |
| Mood swings | 0 1 2 3 | rarely weekly daily | |
| Brain fog | 0 1 2 3 | rarely weekly daily | |
| Poor memory or concentration | 0 1 2 3 | rarely weekly daily | |

# Where I'm starting, continued

| Symptom | Heat scale today | How often | Notes |
|---|---|---|---|
| **Body** | | | |
| Joint pain or stiffness | 0 1 2 3 | rarely weekly daily | |
| Muscle aches | 0 1 2 3 | rarely weekly daily | |
| Headaches or worse migraines | 0 1 2 3 | rarely weekly daily | |
| Weight gain around the middle | 0 1 2 3 | rarely weekly daily | |
| Breast tenderness | 0 1 2 3 | rarely weekly daily | |
| Bloating or digestive changes | 0 1 2 3 | rarely weekly daily | |
| **Periods** | | | |
| Irregular periods | 0 1 2 3 | rarely weekly daily | |
| Heavier bleeding | 0 1 2 3 | rarely weekly daily | |
| Spotting between periods | 0 1 2 3 | rarely weekly daily | |
| **Vaginal, bladder and sex** | | | |
| Vaginal dryness, itching or burning | 0 1 2 3 | rarely weekly daily | |
| Pain during sex | 0 1 2 3 | rarely weekly daily | |
| Lower sex drive | 0 1 2 3 | rarely weekly daily | |
| Needing to pee more often or urgently | 0 1 2 3 | rarely weekly daily | |
| Leaking urine | 0 1 2 3 | rarely weekly daily | |
| UTI-like symptoms | 0 1 2 3 | rarely weekly daily | |
| **Skin, hair and mouth** | | | |
| Dry or itchy skin | 0 1 2 3 | rarely weekly daily | |
| Thinning hair | 0 1 2 3 | rarely weekly daily | |
| Sensitive teeth or sore gums | 0 1 2 3 | rarely weekly daily | |
| **Anything else** | | | |
| | 0 1 2 3 | rarely weekly daily | |
| | 0 1 2 3 | rarely weekly daily | |

## The three that affect my life most

1 _____

2 _____

3 _____

# Period and bleeding calendar

Write the month in the first column, then mark each day you bleed. Changes in your cycle are one of the clearest signs of where you are in the transition.

**Key:** S spotting  L light  M medium  H heavy  X very heavy or flooding

| Month | 1 | 2 | 3 | 4 | 5 | 6 | 7 | 8 | 9 | 10 | 11 | 12 | 13 | 14 | 15 | 16 | 17 | 18 | 19 | 20 | 21 | 22 | 23 | 24 | 25 | 26 | 27 | 28 | 29 | 30 | 31 |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
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# Weekly tracker



Week starting _____

| | Mon | Tue | Wed | Thu | Fri | Sat | Sun |
|---|---|---|---|---|---|---|---|
| **Daily check** | | | | | | | |
| Hot flashes number today | | | | | | | |
| Night sweats 0 to 3 | | | | | | | |
| Hours of sleep | | | | | | | |
| Sleep trouble 0 to 3 | | | | | | | |
| Low mood 0 to 3 | | | | | | | |
| Anxiety or irritability 0 to 3 | | | | | | | |
| Brain fog 0 to 3 | | | | | | | |
| Tiredness 0 to 3 | | | | | | | |
| Joint or muscle pain 0 to 3 | | | | | | | |
| Headache 0 to 3 | | | | | | | |
| Vaginal or bladder symptoms 0 to 3 | | | | | | | |
| Bleeding S, L, M, H or X | | | | | | | |
| **Possible triggers** | | | | | | | |
| Alcohol drinks | | | | | | | |
| Caffeine after noon yes or no | | | | | | | |
| Stress 0 to 3 | | | | | | | |
| Movement or exercise minutes | | | | | | | |
| **Treatment** | | | | | | | |
| Medicine or treatment taken tick | | | | | | | |
| | | | | | | | |
| | | | | | | | |

## This week's notes

## Best and hardest day: what was different?

# Weekly tracker



Week starting _____

| | Mon | Tue | Wed | Thu | Fri | Sat | Sun |
|---|---|---|---|---|---|---|---|
| **Daily check** | | | | | | | |
| Hot flashes number today | | | | | | | |
| Night sweats 0 to 3 | | | | | | | |
| Hours of sleep | | | | | | | |
| Sleep trouble 0 to 3 | | | | | | | |
| Low mood 0 to 3 | | | | | | | |
| Anxiety or irritability 0 to 3 | | | | | | | |
| Brain fog 0 to 3 | | | | | | | |
| Tiredness 0 to 3 | | | | | | | |
| Joint or muscle pain 0 to 3 | | | | | | | |
| Headache 0 to 3 | | | | | | | |
| Vaginal or bladder symptoms 0 to 3 | | | | | | | |
| Bleeding S, L, M, H or X | | | | | | | |
| **Possible triggers** | | | | | | | |
| Alcohol drinks | | | | | | | |
| Caffeine after noon yes or no | | | | | | | |
| Stress 0 to 3 | | | | | | | |
| Movement or exercise minutes | | | | | | | |
| **Treatment** | | | | | | | |
| Medicine or treatment taken tick | | | | | | | |
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## This week's notes

## Best and hardest day: what was different?

# Weekly tracker



Week starting _____

| | Mon | Tue | Wed | Thu | Fri | Sat | Sun |
|---|---|---|---|---|---|---|---|
| **Daily check** | | | | | | | |
| Hot flashes number today | | | | | | | |
| Night sweats 0 to 3 | | | | | | | |
| Hours of sleep | | | | | | | |
| Sleep trouble 0 to 3 | | | | | | | |
| Low mood 0 to 3 | | | | | | | |
| Anxiety or irritability 0 to 3 | | | | | | | |
| Brain fog 0 to 3 | | | | | | | |
| Tiredness 0 to 3 | | | | | | | |
| Joint or muscle pain 0 to 3 | | | | | | | |
| Headache 0 to 3 | | | | | | | |
| Vaginal or bladder symptoms 0 to 3 | | | | | | | |
| Bleeding S, L, M, H or X | | | | | | | |
| **Possible triggers** | | | | | | | |
| Alcohol drinks | | | | | | | |
| Caffeine after noon yes or no | | | | | | | |
| Stress 0 to 3 | | | | | | | |
| Movement or exercise minutes | | | | | | | |
| **Treatment** | | | | | | | |
| Medicine or treatment taken tick | | | | | | | |
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## This week's notes

## Best and hardest day: what was different?

# Weekly tracker



Week starting _____

| | Mon | Tue | Wed | Thu | Fri | Sat | Sun |
|---|---|---|---|---|---|---|---|
| **Daily check** | | | | | | | |
| Hot flashes number today | | | | | | | |
| Night sweats 0 to 3 | | | | | | | |
| Hours of sleep | | | | | | | |
| Sleep trouble 0 to 3 | | | | | | | |
| Low mood 0 to 3 | | | | | | | |
| Anxiety or irritability 0 to 3 | | | | | | | |
| Brain fog 0 to 3 | | | | | | | |
| Tiredness 0 to 3 | | | | | | | |
| Joint or muscle pain 0 to 3 | | | | | | | |
| Headache 0 to 3 | | | | | | | |
| Vaginal or bladder symptoms 0 to 3 | | | | | | | |
| Bleeding S, L, M, H or X | | | | | | | |
| **Possible triggers** | | | | | | | |
| Alcohol drinks | | | | | | | |
| Caffeine after noon yes or no | | | | | | | |
| Stress 0 to 3 | | | | | | | |
| Movement or exercise minutes | | | | | | | |
| **Treatment** | | | | | | | |
| Medicine or treatment taken tick | | | | | | | |
| | | | | | | | |
| | | | | | | | |

## This week's notes

## Best and hardest day: what was different?

# Weekly tracker



Week starting _____

| | Mon | Tue | Wed | Thu | Fri | Sat | Sun |
|---|---|---|---|---|---|---|---|
| **Daily check** | | | | | | | |
| Hot flashes number today | | | | | | | |
| Night sweats 0 to 3 | | | | | | | |
| Hours of sleep | | | | | | | |
| Sleep trouble 0 to 3 | | | | | | | |
| Low mood 0 to 3 | | | | | | | |
| Anxiety or irritability 0 to 3 | | | | | | | |
| Brain fog 0 to 3 | | | | | | | |
| Tiredness 0 to 3 | | | | | | | |
| Joint or muscle pain 0 to 3 | | | | | | | |
| Headache 0 to 3 | | | | | | | |
| Vaginal or bladder symptoms 0 to 3 | | | | | | | |
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## This week's notes

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## This week's notes

## Best and hardest day: what was different?

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## This week's notes

## Best and hardest day: what was different?

# Hot flash and night sweat log

Use this for a week or two when hot flashes are frequent. Logging a few in detail shows patterns a daily count can miss.

| Date | Time | Day or night | Heat scale | How long | What was happening | What helped |
|---|---|---|---|---|---|---|
| | | day night | 0 1 2 3 | | | |
| | | day night | 0 1 2 3 | | | |
| | | day night | 0 1 2 3 | | | |
| | | day night | 0 1 2 3 | | | |
| | | day night | 0 1 2 3 | | | |
| | | day night | 0 1 2 3 | | | |
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| | | day night | 0 1 2 3 | | | |
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| | | day night | 0 1 2 3 | | | |
| | | day night | 0 1 2 3 | | | |
| | | day night | 0 1 2 3 | | | |

# Trigger detective

After four weeks, use your weekly trackers to compare days with and without each possible trigger.

| Possible trigger | Days with it | Avg hot flashes on those days | Days without it | Avg hot flashes on those days | Pattern? |
|---|---|---|---|---|---|
| Alcohol | | | | | yes no |
| Caffeine after noon | | | | | yes no |
| Spicy food | | | | | yes no |
| Hot drinks | | | | | yes no |
| High stress | | | | | yes no |
| Warm room or bedding | | | | | yes no |
| Poor sleep the night before | | | | | yes no |
| | | | | | yes no |

## How to work out an average

Add up the hot flashes on the days you had the trigger, then divide by the number of those days. Do the same for the days without it. A clear difference across several weeks is worth mentioning to your clinician.

### An honest note

Expert reviews haven't found that avoiding triggers or cooling down reduces how often hot flashes happen overall, so they aren't recommended as treatments. Some women still find them helpful for comfort, and knowing your own pattern is useful. If symptoms are affecting your life, treatments with stronger evidence are worth discussing. See "Know your options". [4]

## What I noticed

# Monthly review



Take ten minutes at the end of each month. Look back through your weekly trackers and summarise what you see.

| My main symptoms | Start of month | End of month | Better, same or worse? |
|---|---|---|---|
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |

Avg hot flashes a day _____ Night sweat nights a week _____ Avg hours of sleep _____

My bleeding pattern this month _____

## What helped

## What made things worse

## Questions that came up

# Monthly review

Take ten minutes at the end of each month. Look back through your weekly trackers and summarise what you see.

| My main symptoms | Start of month | End of month | Better, same or worse? |
|---|---|---|---|
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |

Avg hot flashes a day _____ Night sweat nights a week _____ Avg hours of sleep _____

My bleeding pattern this month _____

## What helped

## What made things worse

## Questions that came up

# What I've tried

Record every treatment, product or change you try, including ones that didn't help. It saves time and repeat experiments later.

| Treatment or change | Started | As prescribed or directed | Before | After 4 to 12 weeks | Side effects, or why I stopped |
|---|---|---|---|---|---|
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
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| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |
| | | | 0 1 2 3 | 0 1 2 3 | |

Rate your most bothersome symptom in the before and after columns. Don't change the dose of a prescribed medicine without talking to your prescriber.

# Prepare for your appointment

Most appointments are short. Deciding what matters most before you go makes it far more likely you'll leave with a plan.

Appointment date and time _____ Clinician _____

## My top three concerns

**1**

**2**

**3**

## What I'd like from this visit

- ☐ Understand what's causing my symptoms
- ☐ Discuss hormone therapy
- ☐ Discuss non-hormonal options
- ☐ Help with sleep
- ☐ Help with mood
- ☐ Help with vaginal or bladder symptoms
- ☐ Check my bone and heart health
- ☐ A referral to a menopause specialist

## Bring with you

- ☐ This workbook, or the one-page summary
- ☐ Your medicines and supplements list
- ☐ Your bleeding calendar
- ☐ Your ticked questions
- ☐ Insurance card and ID
- ☐ Someone to support you, if that helps

Ask for a longer or menopause-focused appointment when you book. The Menopause Society has a directory of certified practitioners at menopause.org.

# My symptom summary

Fill this in from your tracking, then hand it to your clinician at the start of the appointment.

Name _____ Age _____

Weeks tracked _____ First day of last period _____

My cycle lately _____

## Main symptoms

| Symptom | When I started tracking | Now | How often |
|---|---|---|---|
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |
| | 0 1 2 3 | 0 1 2 3 | |

Avg hot flashes a day _____ Night sweat nights a week _____ Avg hours of sleep _____

## How this is affecting me

- ☐ Work
- ☐ Sleep
- ☐ Relationships
- ☐ Sex life
- ☐ Mood
- ☐ Exercise

In my words _____

## What I've already tried

_____

_____

_____

## Relevant history

Blood clots, stroke, heart disease, breast or womb cancer, liver disease, migraine with aura

_____

_____

## What I'm hoping for today

_____

_____

Prepare

# Questions to ask

Tick the three to five that matter most to you. Ask them first, in case time runs short.

## About my symptoms

- ☐ Could my symptoms be perimenopause or menopause?
- ☐ What else could be causing them, and should we test for anything (for example thyroid or iron levels)?
- ☐ Do I need any blood tests to confirm where I am?

## About hormone therapy

- ☐ Am I a good candidate, given my age and history?
- ☐ What are my own benefits and risks, in actual numbers?
- ☐ Which form would suit me: patch, gel, spray or pill?
- ☐ Do I need a progestogen, and which one?
- ☐ How soon should I notice a difference, and when will we review it?
- ☐ Which side effects are normal in the first few months?
- ☐ How long can I take it, and how would I stop?

## About non-hormonal options

- ☐ Which non-hormonal medicines could help my hot flashes?
- ☐ What monitoring would I need, such as liver tests?
- ☐ Could CBT help my sleep or hot flashes, and how do I find it?

### Answers and notes

# Questions to ask, continued

## Vaginal, bladder and sex

- ☐ Would a moisturiser, lubricant or low-dose vaginal estrogen help me?
- ☐ Can I use vaginal estrogen alongside other treatment?
- ☐ Could pelvic floor physiotherapy help with leaks?

## Mood and sleep

- ☐ Could my mood changes be related to menopause, and what are my options?
- ☐ Should I be screened for depression or anxiety?
- ☐ What can help with waking at night?

## Long-term health

- ☐ Should I have a bone density scan?
- ☐ Which heart health checks do I need (blood pressure, cholesterol, blood sugar)?
- ☐ Which screenings am I due for?

## Next steps

- ☐ When should I come back?
- ☐ What should make me call sooner?
- ☐ If this doesn't help, can you refer me to a menopause specialist?

### My own questions

# Say it clearly in a short appointment

Fill in the blanks before you go. Saying your main point in the first minute sets the agenda for the whole visit.

## Open with this

"I'm here because of _____ that started around _____. They're affecting my _____. I've tracked them for _____ weeks, and here's a one-page summary. Today I'd like to _____."

## If you feel brushed off

You can stay polite and still be clear. These phrases help keep the conversation going:

- "I understand my tests may be normal, but these symptoms are affecting my life. What can we try?"
- "What would you suggest if hormone therapy isn't right for me?"
- "Could you note in my record that I asked about treatment, and the reason it isn't suitable right now?"
- "Is there a menopause specialist you could refer me to?"
- "Can we book a follow-up to review this in three months?"

## Before you leave

Repeat the plan back in your own words: "So the plan is _____, and I'll come back in _____." It catches misunderstandings while you're still in the room.

## If you want a second opinion

It's reasonable to ask for one, especially if your symptoms are affecting your work, sleep or relationships. A clinician with menopause training, such as a Menopause Society Certified Practitioner, may offer options a general appointment didn't cover.

### Practise your opening here

# Appointment notes



Date _____ Clinician _____

## What they think is going on

## Tests ordered, and when I'll get results

## Treatment plan

| Name | Dose and how to take it | Side effects to watch for |
|---|---|---|
| | | |
| | | |
| | | |
| | | |

## Other advice

Follow-up date _____ Call sooner if _____

## Questions I didn't get to

# Appointment notes



Date _____ Clinician _____

## What they think is going on

## Tests ordered, and when I'll get results

## Treatment plan

| Name | Dose and how to take it | Side effects to watch for |
|---|---|---|
| | | |
| | | |
| | | |
| | | |

## Other advice

Follow-up date _____ Call sooner if _____

## Questions I didn't get to

# After the visit

Most treatments take weeks to show their full effect. Check in at set points, so you and your clinician can judge what's working.

Treatment or change I started _____

Start date _____ Review appointment _____

| My main symptoms | Before | Week 4 | Week 12 |
|---|---|---|---|
| | 0 1 2 3 | 0 1 2 3 | 0 1 2 3 |
| | 0 1 2 3 | 0 1 2 3 | 0 1 2 3 |
| | 0 1 2 3 | 0 1 2 3 | 0 1 2 3 |
| | 0 1 2 3 | 0 1 2 3 | 0 1 2 3 |
| | 0 1 2 3 | 0 1 2 3 | 0 1 2 3 |

## Side effects I've noticed

## At my review, I want to

- ☐ Continue as is
- ☐ Adjust the dose or form
- ☐ Try something different
- ☐ Add something for a symptom that hasn't improved
- ☐ Get a referral

Tracking again for two weeks before your review gives your clinician a clear before-and-after picture. Print extra copies of the weekly tracker pages whenever you need them.

# Sources

Every medical statement in this workbook comes from one of these sources, checked in November 2026.


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## About MenoVerdict

MenoVerdict is an independent guide for women in perimenopause and menopause. We grade the evidence behind treatments and products, link every source, and say plainly when something isn't supported. Find articles, tools and updates at menoverdict.com.

## Using this workbook

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